



Did you know...

A birthing chair may sound like something out of Victorian times, but they are a great alternative to lying in a bed if you're tired. Gravity can assist your labor, but you don't have to use stamina to stay upright. Modern birthing chairs are slightly reclined, padded armchairs. They have a U-shaped cut-out in the seat for your doctor to examine you and for the baby to pass through. A birthing stool is similar, but because it has no back, your birth partner can sit behind you, holding, massaging, and reassuring you.

is the next best thing. Studies show that giving birth in this position reduces the risk of episiotomy, assisted birth, and lowered heart rate in your baby when compared with lying on your back. Lying on your side also helps to open your pelvis (as with squatting), which makes it easier for your baby to pass through.

Q If I am attached to a heart monitor, must I lie down or can I sit up?

As long as your position doesn't interfere with the electrodes and wires that are attached to your belly, you can sit up if you prefer. You can even kneel on all fours, or kneel upright leaning on your birth partner's shoulders as he or she stands by the bed. Being on a bed doesn't necessarily mean you have to lie down.

Q Will having an epidural mean that I have to give birth lying down?

You will probably need to stay in bed if you have an epidural because you will lose the sensation in your body from the waist down.

Some hospitals will carefully monitor the administration of the epidural drug, and as you near the pushing stage they will allow it to wear off, with your consent. This will help you to be guided by your own body during the moments your baby is being born. Your doctor will advise you as to which positions you may be able to manage safely with support.

Q Does every woman tear, and will I need stitches if I do?

Around 85 percent of women will have some perineal trauma (injury) during birth, accounting for both tears and episiotomies (see below). Tears are graded according to the amount of trauma they cause.

» **A first-degree tear** is a superficial wound to your vagina, damaging only a small area of the vagina's skin and fat layer and it probably won't require any stitching.

» **A second-degree tear** is deeper, extending into the muscle of your perineum. Your doctor will probably stitch the tear and you may need pain medicine to ease the soreness for a few days.

» **A third-degree tear** extends into the anal sphincter, causing significant trauma. This type

of tear needs to be repaired by a doctor in an operating room, usually under epidural or spinal anesthesia. The wound can take several weeks to heal, and you may experience some fecal incontinence while everything knits back together, but rest assured that this should only be a temporary effect.

» **A fourth-degree tear** is uncommon and extends into not only the anal sphincter, but also the muscular wall of your anal canal. Usually this kind of tear requires stitching in an operating room. Again, the wound can take weeks to heal and you may suffer some temporary fecal incontinence. Your doctor and a physical therapist will talk to you about how you can help the damage to heal and strengthen any damaged muscles so that your recovery is as quick as possible. If at any point the soreness seems to get worse or the area around the stitches becomes inflamed, let your doctor know in case it has become infected.

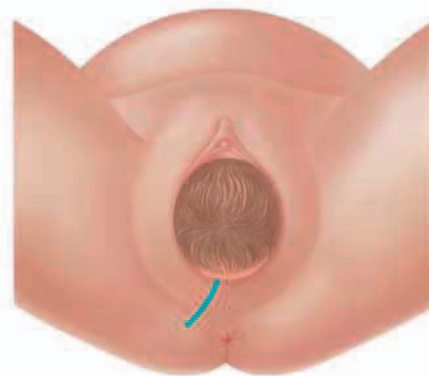
Q What is an episiotomy and why might I need one?

An episiotomy is a surgical incision that opens the perineum, allowing your baby an easier passage, and you will be given one only if there is a sound clinical reason.

An episiotomy enables the doctor to control your baby's exit, but this will not be done routinely, and you will need to give your verbal consent before an incision. Your doctor will explain the reasons why the procedure is being recommended. Among those reasons are that your perineum is too rigid to stretch to allow your baby through, putting you at increased risk of a third- or fourth-degree tear (see above); to ease the delivery if you have a medical condition that makes the effort of giving birth dangerous for you, for example, if you have a heart condition or high blood pressure; if your baby is showing signs of distress (a rapid or a weak heartbeat); or if your baby is stuck and you need an assisted birth (see p.222).

The cut itself is about 3–4cm long and made with round-ended surgical scissors or with a scalpel as your baby descends into the perineum. You will have a local anaesthetic to numb the area both while the cut is being made and later when you're being stitched. The stitching will usually be done within an

hour of the end of the delivery of your baby. Once the anesthetic has worn off, you are likely to feel sore for about a week while the incision heals, and you may have a stinging sensation when you urinate. Pouring warm water over the area while you urinate can help to reduce the stinging sensation.



A diagonal episiotomy The cut is usually made from the back of the vagina, then out to one side. Dissolvable stitches are used to repair it.